DRAFT – NOT FOR FILING

5122-29-01 Purpose and applicability.

The purpose of this chapter is to state the requirements for the provision of behavioral health services by a provider, as that term is defined in paragraph (A) of rule 5122-25-01 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-03 General services.

(A) As used in this rule, "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25-01 of the Administrative Code applies.

(B) "General services" are the assessment activities, medical activities, and counseling and therapy activities described in this rule.

(C) General services are to be provided by the professionals specified in appendix A to this rule within the scope of practice of those professionals. If such professionals are providing general services to individuals with a substance use disorder, their delivery of the services are subject to the following:

(1) If the services to be provided are level 2.1 services under American society of addiction medicine (ASAM) criteria, third edition, 2013, known as intensive outpatient services, the services are to be provided in accordance with that criteria.

(2) If the services to be provided are level 2.5 services under ASAM criteria, third edition, 2013, known as partial hospital services, the services are to be provided in accordance with that criteria.

(D) Assessment activities

(1) Definition

An assessment is an individualized, person-centered, developmentally appropriate, and culturally responsive clinical evaluation of an individual, conducted by qualified staff, that determines an individual's diagnosis and treatment needs and directly informs the development of the individual's initial and comprehensive individualized treatment plans described in rule 5122-27-03 of the Administrative Code to address the individual's primary or co-occurring mental illness and/or substance use disorder, problem gambling, tobacco or nicotine use.

(2) Timing

When an individual is to receive an assessment, it should begin before any other services are provided, unless the situation is an emergency.

(3) Initial assessment standards

An initial assessment is to include, at a minimum, an evaluation of all of the following:

(a) The presenting problem;

(b) The risk of harm to self and others;

(c) The use of alcohol or drugs;

(d) History of treatment for mental illness, substance use, problem gambling, or tobacco or nicotine use; and,

(e) A medical history and examination (mental status or physical).

(4) Comprehensive assessment standards

A comprehensive assessment expands on the initial assessment by gathering any additional information needed to establish and implement a comprehensive treatment plan. It is to be completed within thirty days of the initial assessment encounter.

(5) Order of assessments

It is not necessary that an individual have an initial assessment before receiving a comprehensive assessment.

(6) Standards of care

Both initial and comprehensive assessments for mental health and substance use disorder are to follow prevailing standards of care and accreditation guidelines specified by:

(a) The joint commission;

(b) The commission on accreditation of rehabilitation facilities;

(c) The council on accreditation;

(d) The American society of addiction medicine (ASAM); or

(e) Other entities as designated by the director.

(7) Use of prior assessments

A provider may accept an initial or comprehensive assessment completed by another provider within the past twelve months. The prior assessments are to be reviewed and updated as necessary.

(E) Counseling and therapy

(1) Definition

Counseling and therapy is an interaction with an individual or group of individuals that focuses on achieving treatment objectives related to mental illness and/or substance use disorder, problem gambling, tobacco or nicotine use, and/or co-occurring conditions, as indicated.

(2) Format

Counseling and therapy involves interaction between a behavioral health professional and an individual, group of individuals, individual and family members, or family members and a behavioral health professional.

(3) Ratio of behavioral health professionals to clients

Group counseling and therapy encounters are to maintain a ratio no greater than one behavioral health professional to every twelve clients.

(F) Medical activities.

(1) Definition:

Medical activities are those activities that meet both of the following criteria:

(a) Are performed within the professional scope of practice by an individual who is licensed or certified by the state medical board of Ohio or the Ohio board of nursing, or who is a pharmacist licensed by the Ohio board of pharmacy; and

(b) Are intended to address the behavioral and other physical health needs of clients receiving treatment for psychiatric symptoms or substance use disorders.

(2) Examples

Medical activities include, but are not limited to:

(a) Performing health care screenings, assessments, and exams;

(b) Checking vital signs;

(c) Ordering laboratory tests and reviewing the results; and

(d) Medication prescribing, administering, and monitoring.

DRAFT – NOT FOR FILING

5122-29-06 Mental health day treatment service.

(A) As used in this rule:

(1) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(2) "Mental health day treatment program day" means the total amount of hours an individual receives mental health day treatment service during a twenty-four hour calendar day.

(B) Mental health day treatment is an intensive, structured, goal-oriented, distinct, and identifiable treatment service that utilizes multiple mental health interventions that address the individualized mental health needs of the client. Mental health day treatment services are clinically indicated by assessment with clear admission and discharge criteria. The environment at this level of treatment is highly structured and there should be an appropriate staff-to-client ratio to guarantee sufficient therapeutic services and professional monitoring.

The purpose and intent of mental health day treatment is to stabilize, increase, or sustain the highest level of functioning and promote movement to the least restrictive level of care.

The outcome is for the individual to develop the capacity to continue to work towards an improved quality of life with the support of an appropriate level of care.

(C) Mental health day treatment is an intense treatment service that consists of high levels of face-to-face mental health interventions that address an individual's mental health needs as identified in the individual's individualized treatment plan (ITP).

(D) The minimum program length of this service is to be in accordance with the appropriate behavioral health standards of the agency's one or more national accrediting bodies. Such accrediting bodies are identified in rule 5122-25-02 of the Administrative Code.

(E) For purposes of this rule, a mental health day treatment program day is to consist of a minimum of two hours and up to a maximum of seven hours per day, at least three days a week, of scheduled intensive activities that may include, but are not limited to, the following:

(1) Determination of needed mental health interventions;

(2) Skills development

(a) Interpersonal and social competency as age, developmentally, and clinically appropriate, such as:

(i) Functional relationships with adults;

(ii) Functional relationships with peers;

(iii) Functional relationships with the community/schools;

(iv) Functional relationships with employer/family; and

(v) Functional relationships with authority figures.

(b) Problem solving, conflict resolution, and emotions/behavior management.

(c) Developing positive coping mechanisms;

(3) Managing mental health and behavioral symptoms to enhance vocational/school opportunities and/or independent living; and

(4) Psycho-educational interventions including individualized instruction and training of persons served in order to increase their knowledge and understanding of their psychiatric diagnosis(es), prognosis(es), treatment, and recovery, improve their coping skills, and favorably affect their outcomes. Such education is to be consistent with the individual's ITP and be provided with the knowledge and support of the interdisciplinary/intersystem team providing treatment in coordination with the ITP.

(5) That the provider, as identified in the individual's ongoing assessment, provide, directly or through appropriate referral, treatment related to medical, psychological, or psychiatric needs.

(6) That the provider has the ability to monitor program participants for changes in areas such as symptom severity, the need to move to a higher or lower level of care, and shifts in social skills or daily functioning. This ongoing monitoring helps identify early signs of clinical decline, guide timely interventions, assess how well treatments are working, adjust the comprehensive individualized treatment plan developed under rule 5122-27-03 of the Administrative Code, and support each program participant's recovery goals by strengthening coping skills, encouraging community engagement, and promoting independence in daily living.

(F) Providers of mental health day treatment services are to have a staff development plan based upon identified individual needs of mental health day treatment program staff. Evidence that the plan is being followed shall be maintained.

(G) Mental health day treatment service is to be provided and supervised by an interdisciplinary team of mental health treatment staff who are qualified according to rule 5122-29-30 of the Administrative Code, and only those components of the service which are appropriate to the individual's credential or qualifications.

DRAFT – NOT FOR FILING

5122-29-07 Forensic evaluation service.

(A) "Forensic evaluation service" means an evaluation resulting in a written expert opinion regarding a legal issue for an individual referred by a criminal court, domestic relations court, juvenile court, probate court, adult parole authority, or other agency of the criminal justice system or a department-operated regional psychiatric hospital. Forensic evaluation service includes all related case consultation and expert testimony. Forensic evaluation service also assists courts and the adult parole authority to address behavioral health legal issues such as those referenced in paragraph (B) of this rule.

(B) Forensic evaluation service addresses behavioral health legal issues, including, but not limited to, the following:

(1) Competency to stand trial, as defined in division (H)(3) of section 2945.371 of the Revised Code;

(2) Mental condition at the time of the offense charged, as defined in division (H)(4) of section 2945.371 of the Revised Code;

(3) Post-NGRI (not guilty by reason of insanity) examination, as defined in division (A) of section 2945.40 of the Revised Code:

(4) Presentence, as defined in section 2951.03 of the Revised Code;

(5) Mitigation of penalty, as defined in section 2947.06 of the Revised Code;

(6) Mitigation of death penalty, as defined in section 2929.03 of the Revised Code;

(7) Domestic violence evaluation, as defined in section 2919.271 of the Revised Code;

(8) Competence to be a witness, as defined in section 2317.01 of the Revised Code;

(9) Adult parole authority, for parole revocation and other legal questions;

(10) Psychological effects of an act upon the victim, as defined in section 2930.13 of the Revised Code;

(11) Domestic relations, for custody and visitation;

(12) Juvenile dependency, neglect, delinquency (Ohio rules of juvenile procedure, rule 32); competency as defined in section 2152.53 of the Revised Code; or waiver to adult court, as defined in division (B) of section 2152.12 of the Revised Code;

(13) Battered woman syndrome, as defined in section 2945.392 of the Revised Code;

(14) Violation of anti-stalking protection order, as defined in section 2903.212 of the Revised Code;

(15) Intervention in lieu of conviction, as defined in section 2951.041 of the Revised Code;

(16) Non-secured status, as defined in section 2945.401 of the Revised Code;

(17) Post sentence evaluation-probation or parole for involuntary commitment, as defined in section 2967.22 of the Revised Code;

(18) Juvenile competency evaluation for serious youthful offenders, as defined in division (C) of section 2152.13 of the Revised Code.; or

(19) Competency to waive Miranda rights.

(C) No examiner should undertake a forensic evaluation without at least one of the following:

(1) An appropriate written order from the court ordering the evaluation;

(2) An official written request if the agency requesting the forensic evaluation is a parole or probation department; or

(3) A department-operated regional psychiatric hospital.

(D) Forensic evaluation service is to provide the following standards of confidentiality:

(1) The relationship between the individual being evaluated and the examiner is not confidential in the usual understanding of that term. A written report is to be made to the court or adult parole authority, whether or not the individual being evaluated cooperates with the examiner. The relationship between the examiner, evaluee, and court or adult parole authority is to be explained orally in terms that the individual can understand, and this process and the level of understanding is documented in the report and may also be presented in writing to the individual being evaluated. It is to be clearly noted that information gathered and expert opinions reached by the examiners is to be summarized in a written report and/or testimony to the court or adult parole authority or other referring agency.

(2) Reports to the criminal courts are to be forwarded only to the court that referred the individual or to other court officials, prosecution and defense attorneys, as designated by the referring court. The court may, at its discretion, distribute the report, and bears the responsibility for that distribution. Reports to the adult parole authority are to be forwarded only to that agency, which may, at its discretion, distribute the report, and bears the responsibility for that distribution. Reports may be distributed to other parties only with the written authorization of the court adult parole authority, or other referring agency.

(3) Reports of forensic evaluations are to be stored separately from other types of client records, and are to be considered the property of the court that ordered them or the agency that referred the individual.

(E) Each forensic evaluation report is to include at least the following:

(1) The name and qualifications of the examiner(s);

(2) The name of the court or agency that referred the individual;

(3) The legal or referral question being assessed;

(4) Identifying information about the individual being evaluated, including relevant clinical, social, and criminal history;

(5) The duration and location of each interview with the individual being evaluated, including whether the evaluation was conducted in person or through electronic means;

(6) A description of collateral information used to develop the report;

(7) Psychological and/or psychiatric data that address the legal or referral issue, if applicable; and

(8) Opinions and recommendations.

(F) The forensic evaluation report is to be presented in non-technical terms and in reasonable detail. The data and recommendations are to be pertinent to the legal or other referral question. Relevant collateral information is to be used in a forensic evaluation to the fullest extent possible. Opinions in a forensic evaluation report are not to be based entirely on self-report of the individual being evaluated if collateral information is available.

(G) Reports are to contain sufficient information to substantiate the conclusions and recommendations made. Special caution is to be exercised with self-incriminating statements by the individual being evaluated, information about others not being evaluated, or other material of a particularly sensitive, personal nature not related to the issue and for which the forensic evaluation was requested.

(H) For competence to stand trial and not guilty by reason of insanity forensic evaluations, the qualifications of the examiner(s) are regulated by sections 2945.37 and 2945.371 of the Revised Code. All other examinations for which qualifications are not specified by law are to be conducted by staff who are qualified according to paragraph (K) of this rule.

(I) Forensic evaluations are to be completed within the time limits specified by law, unless an extension has been granted by the referral source. Examinations for which no statutory time limit exists are to be completed within a reasonable time, as determined in consultation with the court or agency requesting the service.

(J) The agency is to ensure that:

(1) All staff who perform forensic evaluation services have training and continuing education relating to the legal and behavioral health issues involved in the services they provide; and

(2) All persons who perform forensic evaluation services listed in paragraph (B) of this rule provide written documentation of at least twenty-four hours of training every three calendar years that is specific to the forensic behavioral health area.

(K) Forensic evaluation service is to be provided and supervised by staff who are qualified according to rule 5122-29-30 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-08 Behavioral health hotline service.

(A) Behavioral health hotline service means a provider's twenty-four hours per day, seven days per week capability to respond to telephone calls, often anonymous, made to a provider for crisis assistance. The caller may or may not be a client of the provider.

(B) Behavioral health hotline service shall:

(1) Staff the service so that calls are answered twenty-four hours per day, seven days per week;

(2) Provide referrals to crisis intervention service;

(3) Include, but not be limited to, the following:

(a) The provision of support, intervention, and crisis management by telephone to individuals in crisis;

(b) Engagement in suicide prevention intervention, including inquiring if the individual has a crisis safety plan and using this information in the intervention;

(c) The provision of appropriate linkages to all needed services and other community resources, including peer support services as applicable;

(d) The provision of information regarding crisis services, including the local crisis center phone number and additional referral to support services as indicated; and,

(e) The provision of information and referral to immediate psychiatric and medical services when indicated, such as the crisis center or a hospital emergency room.

(4) Ensure that all staff and volunteers receive training in crisis intervention techniques, safety planning, management of risk, and available resources and supports in the county or region where the provider is located;

(5) Be provided by staff and volunteers qualified according to paragraph (D) of this rule; and,

(6) Document the call in the client record if it is known that the caller is an individual served by the provider.

(C) The provider service plan for behavioral health hotline services is to include, but not be limited to, the requirements that the service:

(1) Function as part of an integrated, comprehensive system of health, behavioral health, and other human service providers;

(2) Ensure the ability to use and work with case management systems, other involved health care providers, and crisis intervention service on a priority basis;

(3) Coordinate with the community's emergency service systems, such as hospital, crisis centers, fire, police, ambulance services, etc.;

(4) Maintain a current listing of available residential or housing placements that can be accessed quickly when emergency housing is needed in conjunction with a crisis intervention mental health service; and

(5) Is provided as part of the alcohol, drug addiction, and mental health services board's emergency crisis plan for the service district.

(D) Behavioral health hotline service is to be provided and supervised by staff and volunteers who are qualified according to rule 5122-29-30 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-09.1 Substance use disorder qualified residential treatment program (QRTP) for youth.

(A) As used in this rule, "provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) A provider that provides a level three substance use disorder residential or withdrawal management program certified by the department in accordance with rule 5122-29-09 of the Administrative Code and accepts children or adolescents (youth) for placement is to be certified for the service of substance use disorder qualified residential treatment program (QRTP) for youth under this rule.

(C) A provider is to meet the following standards to attain certification for the service of substance use disorder qualified treatment program (QRTP) for youth:

(1) Have a residential program that is accredited by at least one of the following national accrediting bodies and provide ongoing proof of such accreditation status to the department:

(a) Commission on accreditation of rehabilitation facilities;

(b) The joint commission; or

(c) Council on accreditation.

(2) Implement a trauma-informed approach in which all employees, volunteers, interns, and independent contractors within the location of the level three substance use disorder treatment program are trained in that trauma-informed approach. Trauma-informed training is to occur within the first thirty days after the date of hire and annually thereafter. The required trauma competencies are located at https://jfs.ohio.gov/static/ocf/FFPSA-Competence-Rev202201.pdf.

(3) Utilize a trauma-informed treatment model that is approved by the department for the population the agency serves. A trauma-informed treatment model is a program, organization, or system that:

(a) Ensures all clinical staff are trained on the trauma model approved by the department. The facility or agency is to describe in writing in its trauma training policies and procedures or elsewhere whether non-clinical staff will be trained on the trauma model or will be trained only on the trauma competencies described in paragraph (C)(2) of this rule.

(b) Realizes the widespread impact of trauma and understands potential paths for recovery;

(c) Recognizes the signs and symptoms of trauma in clients, families, staff and others involved with the system;

(d) Responds by fully integrating information about trauma into policies, procedures and practices;

(e) Seeks to actively resist re-traumatization;

(f) Includes service of clinical needs and that:

(i) Is an approved trauma informed treatment model applicable to the population of youth served located at https://childrenandyouth.ohio.gov/for-providers/foster-care-licensing/residential-functions/trauma-treatment-model;

(ii) Meets the ten substance abuse and mental health services administration (SAMHSA) implementation domains and follows the six key principles of the SAMHSA trauma informed approach which are described in this SAMHSA document: https://www.nctsn.org/sites/default/files/resources/resource-guide/samhsa_trauma.pdf; and

(iii) Receives approval by the department or the department's designee.

(4) Have registered or licensed nursing and clinical staff who operate in accordance with the following:

(a) Provide care within the scope of their practice as defined by state law;

(b) Are accessible on-site or via interactive videoconferencing based on the youth's clinical or medical needs. Interactive videoconferencing might not be appropriate for a youth in crisis at the agency; and

(c) Are available twenty-four hours a day and, seven days a week.

(5) With consideration to the youth's safety and developmental needs, the treatment should be family-driven with both the youth and the family included in all aspects of care, if in the best interest of the youth. The key components of family-centered residential treatment are to be documented in the youth's record and include the following:

(a) Facilitation of regular contact between the youth and other members of the family including siblings;

(b) Actively involving and supporting families who have a youth placed in the residential facility; and

(c) Providing outreach, ongoing support and aftercare for the youth and the family.

(6) Complete discharge planning that is to include family-based aftercare support. Family-based aftercare support is defined as individualized, community-based, trauma-informed supports that build on treatment gains to promote the safety and

well-being of youth and families, with the goal of preserving the youth in a supportive family environment. The discharge plan is to:

(a) Include planning for aftercare services for all youth discharged from the agency to family-based settings including:

(i) Reunification with family;

(ii) Pre-finalized adoptive family;

(iii) Kinship care;

(iv) Foster care; or

(v) Independent living.

(b) Begin in partnership with the legal custodian or custodial agency no later than the next business day after a youth is admitted to the QRTP.

(c) Be reviewed by the QRTP no less than every thirty calendar days and during every individualized treatment plan (ITP) review as described by rule 5122-27-03 of the Administrative Code. An ITP review is to be conducted at least every ninety calendar days.

(d) Include at least a six-month period of support after discharge, even if the youth reaches the age of majority. The QRTP is exempt from providing aftercare support if the youth's placement is less than fourteen days.

(e) Be provided within the youth or family's community as appropriate to promote the continuity of care for youth.

(f) Be individualized and driven by the youth, the caregivers and the family as appropriate, and include the following:

(i) Monthly contact with the youth and caregivers to promote and maintain engagement and to regularly evaluate the family's needs. Monthly contact may be in person, through interactive videoconferencing, or via phone or other electronic means.

(ii) Coordinate engagement with any applicable community providers serving the youth or family. The QRTP will ensure they make themselves available to the community providers for ongoing consultation, and document the consultation in writing. Documentation should include all resources and supports needed and detail how the resources and supports will be provided.

(iii) Written documentation provided to all participants of the discharge plan prior to discharge with information on how to access additional supports from the QRTP and community providers including contact information and steps required to access each provider.

(7) Conduct a background check for any employee, volunteer, intern, or independent contractor in accordance with rule 5122-26-06(F)(2) of the Administrative Code prior to hire. No employee, volunteer, intern, or independent contractor may be present in the level three substance use disorder treatment program until the provider has reviewed the results of the background check and assured that the individual is eligible to work under rule 5122-30-31 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-10 Crisis intervention service.

(A) As used in this rule, "provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) Crisis intervention service is an interaction with an individual in response to a crisis or emergency situation the individual is experiencing.

(C) Crisis intervention service includes:

(1) An urgent evaluation of the following elements when clinically indicated:

(a) Understanding what happened to initiate the crisis and the individual's response or responses to it;

(b) Risk assessment of lethality, propensity of violence, and medical/physical condition including alcohol or drug use;

(c) Mental status;

(d) Information about the individual's strengths., coping skills, and social support network, including face-to-face contact with family and collateral informants; and

(e) Identification of treatment needs and appropriate setting of care.

(2) The development of a crisis plan to de-escalate the crisis, stabilize the individual, restore safety, provide referrals and linkages to appropriate services, and coordinate with other systems.

(D) A provider of crisis intervention service is to have current certification in first aid and cardio-pulmonary resuscitation (CPR) and be trained in de-escalation techniques.

(E) When an individual appears to be medically unstable, the individual is to be referred to a medical facility or emergency medical service is to be called.

DRAFT – NOT FOR FILING

5122-29-12 Driver intervention program.

(A) A driver intervention program is a program of screening, education, and referral for individuals who are arrested or convicted of operation of a vehicle or water craft under the influence of alcohol or a drug of abuse under section 4511.19 or 1547.11 of the Revised Code or a substantially similar municipal ordinance or other alcohol-related traffic statute or ordinance.

(B) No entity may operate, or purport to operate, a driver intervention program in Ohio unless it has received driver intervention program certification from the department.

(C) Except as otherwise provided in this rule, the provisions of this rule apply to all driver intervention programs in Ohio, public or private.

The provisions in this rule do not negate the necessity of driver intervention programs to be certified providers in accordance with the provisions of Chapter 5122-25 of the Administrative Code.

(D) A driver intervention program is to provide to the department all of the following at the time of seeking initial certification under rule 5122-25-02 of the Administrative Code, renewal certification under rule 5122-25-04 of the Administrative Code, or when seeking to update its certification of certifiable services or supports under rule 5122-25-03 of the Administrative Code:

(1) The total number of hours of operation, including the total number of program hours;

(2) If services are provided at a resident camp, a copy of the camp permit issued by the appropriate board of health pursuant to rule 3701-25-02 of the Administrative Code;

(3) If services are provided at a hotel or motel, a copy of the facility's occupancy and use certificate and annual fire inspection report or, if those are not available, a copy of the facility's hotel or motel license issued by the division of state fire marshal in the Ohio department of commerce.

(E) Each driver intervention program is to have either representatives from law enforcement officers, judges, prosecuting and defense attorneys, peer service organizations, social service organizations, and treatment centers on its governing authority or establish an advisory board with such representatives.

If an advisory board is established, both of the following are to be the case:

(1) The board is to meet annually; and

(2) Minutes are to be taken and maintained.

(F) Program administration:

(1) Each driver intervention program is to have a program director who is responsible for the overall day-to-day operation of the driver intervention program. The driver

intervention program director is responsible to the governing authority of the organization. If the driver intervention program is a component of a larger organization, the driver intervention program director may report to the executive director/chief executive officer of the organization, who would then report to the governing authority. A program director is to meet the following qualifications:

(a) Have a bachelor's degree and two years' experience in alcohol and other drug addiction services or an allied profession to include one year as a supervisor;

(b) Have three years' experience in alcohol and other drug addiction services or an allied profession to include a minimum of one year as a supervisor; or

(c) Have three years of experience in business administration to include a minimum of one year as a supervisor.

(2) The position description of the driver intervention program director is to include, at a minimum, the following responsibilities;

(a) Overseeing the day-to-day operations of the driver intervention program;

(b) Developing and implementing the policies and procedures of the driver intervention program;

(c) Developing and revising as necessary, the driver intervention program's education curriculum;

(d) Preparing an annual plan for the operation of the driver intervention program;

(e) Implementing the driver intervention program's quality assurance and improvement activities and findings;

(f) Hiring and terminating driver intervention program staff; and

(g) Ensuring that the driver intervention program is operating in accordance with the department's driver intervention program certification standards.

(G) Each driver intervention program is to maintain both of the following:

(1) An outline of the current education curriculum of the driver intervention program.

(2) A copy of the revenue and expenditure budget for the driver intervention program.

(H) Each driver intervention program is to meet, at a minimum, all of the following accessibility requirements for persons with physical disabilities:

(1) Entrances, hallways and spaces where services are provided, as well as office space for employees are to be accessibile to persons with physical disabilities.

(2) The facility is to have at least one bathroom accessible to persons with physical disabilities.

(3) The facility is to have designated parking space(s) for persons with physical disabilities based on the Americans with disabilities act accessibility guidelines.

(4) The facility is to have at least one drinking fountain accessible to persons with physical disabilities.

(5) Facility is to have at least one telephone that is accessible to persons with physical disabilities.

(6) Each residential driver intervention program is to have at least one shower facility accessible to persons with physical disabilities.

(I) Emergency medical plan and first aid supplies

(1) Each driver intervention program is to have a written emergency medical plan that includes, at a minimum, all of the following:

(a) The current emergency telephone numbers for fire, emergency squad, police and poison control;

(b) The location of first aid supplies at the program site during operation of the driver intervention program;

(c) General instructions for medical emergencies including supervision of clients during the emergency;

(d) General instructions in case of illness of a client; and

(e) A procedure for documenting unusual incidents and notifying families.

(2) A copy of the emergency medical plan is to be conspicuously posted at the program site during the operation of each driver intervention program.

(J) Client records

(1) Each driver intervention program is to have written policies and/or procedures for maintaining a uniform client records system that complies with rule 5122-27-02 of the Administrative Code and includes, at a minimum, all of the following:

(a) A statement that program staff, contract employees, volunteers, and student interns are not to convey to a person outside of the program that an individual attends or receives services from the driver intervention program, or disclose any information identifying a client as an alcohol or other drug services client unless the client consents in writing for the release of information; the disclosure is allowed by a court order; the disclosure is to entities with which the provider has entered into a qualified service organization agreement (QSOA) pursuant to 42 C.F.R. part 2; or the disclosure is made to qualified personnel for a medical emergency, research, audit or program evaluation purposes. The driver intervention program has the authority to deny services if a client refuses consent to the release of information.

(b) A statement that the federal laws and regulations do not protect any threat to commit or any information about a crime committed by a client either at the program or against any individual who works for the driver intervention program.

(c) A statement that the federal laws and regulations do not protect any information about suspected child abuse or neglect from being reported under state law to appropriate state or local authorities.

(d) A statement that disclosure made with the client's written consent is to be consistent with 42 C.F.R. part 2 and include one of the written statements mandated by 42 C.F.R. 2.32.

(e) A policy on the access of client records by clients, staff, and others.

(f) Components of client records and timelines, when applicable, for completing each component.

(g) A policy on the storage of client records that requires records be maintained in accordance with 42 C.F.R. part 2.

(h) A policy on the destruction of client records to include the requirement that records be maintained for at least six years after clients have been discharged from the program. Client records are to be destroyed to maintain client confidentiality as required by state and federal law.

(2) A record is to be maintained for each client of a driver intervention program. Each record is to include, at a minimum, all of the following components:

(a) Identification of client (name of client and/or client identification number);

(b) Client fee agreement;

(c) Consent for services;

(d) Documentation reflecting receipt of the schedule for the driver intervention program being attended by the client;

(e) Documentation reflecting receipt of the driver intervention program rules and/or expectations of clients;

(f) Documentation reflecting receipt of the program's policy on client rights that lists the client rights required by this rule;

(g) Documentation reflecting receipt of the program's client grievance procedure;

(h) Documentation reflecting receipt of a written summary of the federal laws and regulations that indicate the confidentiality of client records are protected as required by 42 C.F.R. part 2;

(i) Intake report;

(j) Identification of at least two screening instruments that were administered to the client and documentation of the results of both tests;

(k) Results and recommendations of the screening;

(l) Recommendations for alcohol and/or drug;

(m) Assessment, if completed by the driver intervention program;

(n) Any recommendations made to a court or other organization;

(o) Date of each group session provided;

(p) Length of each group session provided;

(q) Topic/content of each group session provided;

(r) Client's response/feedback during each group session; and

(s) Disclosure of client information forms, when applicable.

(3) Disclosure of client information forms is to include all of the following information as required by 42 C.F.R. part 2:

(a) Name of program making the disclosure;

(b) Name or title of the individual or the name of the organization to which the disclosure is to be made;

(c) Name of the client;

(d) Purpose of the disclosure;

(e) Type and amount of information to be disclosed;

(f) Original signature of the client or person autorized to give consent;

(g) Date client or authorized person signed the form;

(h) Statement that the consent is subject to revocation at any time except to the extent the program or person who is to make the disclosure has already acted in reliance on it; and

(i) The date, event, or condition upon which the consent will expire, unless revoked before that specified time.

(4) Each client record is to include a program completion report that that is to include, at a minimum, all of the following documentation:

(a) Results and recommendations of the screening.

(b) Any recommendations for alcohol and/or drug assessment.

(c) Any recommendations made to a court or other organization.

(d) Number of hours of driver intervention programming the client attended.

(e) Any referrals made to alcohol and drug addiction treatment programs and any referrals made to other organizations.

(f) Date, signature, and credentials of the program director, services supervisor or program staff of the driver intervention program who makes the recommendation.

(g) Summary of client's participation.

(h)) Disclosure of client information form that is prepared in accordance with 42 C.F.R. part 2 for information released to courts, organizations and/or individuals and for management information reports to the department.

A copy of the report is to be sent to the court or organization that referred the client to the driver intervention program.

(5) A policy and procedure, in accordance with 42 C.F.R. part 2, for tracking clients for a reasonable time following program completion.

(6) If a program maintains electronic client records, the program is to be able to produce hard copies of client records upon legally valid requests and have a written policy and procedure indicating how client original signatures and staff original signatures are obtained and verified for documentation.

(7) If a program discontinues operations or is taken over or acquired by another entity, it is to comply with 42 C.F.R. 2.19, which governs the disposition of records by discontinued programs.

(8) Providers are to maintain treatment records for at least seven years after a client has been discharged from a program or services are no longer provided.

(K) Intake report

(1) An intake report is to be completed for each client at the beginning of the first day of the driver intervention program. Documentation is to include, at a minimum, all of the following:

(a) Client identification number and name;

(b) Prescription and over-the-counter drugs being taken by the client;

(c) Type and amount of any medications brought to the program;

(d) Special dietary requirements;

(e) Known allergies, including but not limited to food and drug reactions;

(f) Pregnancy status;

(g) Special needs; and

(h) Name, address, and telephone number of a person who is to be contacted in the event of an emergency.

(2) Baggage and materials brought to the driver intervention program are to be inspected to ascertain that they do not contain contraband, which includes, at a minimum, illegal drugs, alcohol, or firearms. Documentation is to appear in the intake report.

(3) The intake report is to be dated and signed by the staff member completing the intake report.

(L) Screening

(1) Screening means a preliminary gathering and sorting of information used to determine whether a comprehensive assessment is appropriate.

(2) Each client of a driver intervention program is to be administered at least two evidence-informed screening instruments. Those results are to be recorded in the client's record.

(3) An individual screening interview is to be done with each client to discuss the screening results, recommendations, and referrals made to a referring court or other organization. Screening interviews are to include, at a minimum, all of the following:

(a) Client identification;

(b) Presenting problem and/or precipitating factors leading to the need for screening;

(c) Past and present use of alcohol and other drugs;

(d) History of treatment for alcohol and other drug abuse;

(e) Medical problems;

(f) Legal history;

(g) Recommendations for referral, if applicable, for a comprehensive assessment to determine the extent and severity of alcohol and other drug abuse problems and need for treatment; and

(h) Date, signature, and credentials of program staff who completed the screening.

(M) Referral for assessment

(1) Each driver intervention program is to have a written procedure for making referrals for assessment which requires that a completed release of information be obtained prior to contacting a program.

(2) Each driver intervention program is to have a written policy stating that the basis for making a recommendation to a court or other organization for alcohol and drug addiction comprehensive assessment will include, at a minimum, both of the following:

(a) Results of the two screening instruments and screening interview described in paragraph (L) of this rule; and

(b) Observations of the client during screening, client education on alcohol and drug abuse and addiction, and group sessions.

(3) Each driver intervention program is to have a written policy for submitting a copy of the report to the court or organization that referred the client to the driver intervention program, as requested by the referral source.

(N) A driver intervention program may conduct client assessments. Assessments are to be conducted in accordance with rule 5122-29-03 of the Administrative Code.

(O) Dietary services:

Each driver intervention program that prepares and/or serves meals as part of its daily scheduled activities is to operate its dietary services in accordance with laws, regulations, or ordinances of the state medical board of Ohio, Ohio department of health, and/or the appropriate local health department.

(P) Pharmaceutical services

A driver intervention program is subject to rule 5122-26-15 of the Administrative Code regarding the purchase, receipt, storage, distribution, return, and destruction of medication. Clients are not to be denied driver intervention services solely due to their use of prescribed psychotropic medications.

(Q) Small group sessions

A driver intervention program is to include small group sessions. Small group sessions are not to exceed a staff to client ratio of one to fifteen and the total number of clients in a group session is not to exceed fifteen, regardless of the number of staff.

(R) Active supervision and monitoring of clients

A driver intervention program is to have at least one staff member who is on-site and actively supervising and/or monitoring clients at all times during the program.

(S) Program completion reports

A driver intervention program is to ensure that a program completion report is prepared for each client.

(T) Non-residential driver intervention programs:

(1) Each non-residential driver intervention program is to consist of at least thirteen hours of alcohol and drug addiction programming that includes, at a minimum, all of the following:

(a) One hour of screening and individual contact;

(b) Eight hours of client education on alcohol and drug abuse and addiction including traffic safety education; and

(c) Four hours of small group discussion sessions.

(2) Each non-residential driver intervention program is not to deliver more than eight hours of alcohol and drug addiction programming to clients each day.

(3) If a non-residential driver intervention program is operated within a residential driver intervention program, its programmatic content is to be the same as that of the residential driver intervention program and include the mid-day and evening meals.

(4) If a non-residential driver intervention program operates its program for five hours or more on any day, the program is to have provisions for a mid-day meal of at least thirty minutes. This mid-day meal is to be included in the program's schedule that is available to clients upon request. Time for meals is not to supplant any of the minimum thirteen-hour alcohol and drug programming.

(U) Forty-eight hour residential driver intervention programs

Each forty-eight hour residential driver intervention program is to consist of at least sixteen hours of alcohol and drug addiction programming that includes, at a minimum, all of the following:

(1) One hour of screening and individual contact;

(2) Ten hours of client education on alcohol and drug abuse and addiction including traffic safety education; and

(3) Five hours of small group discussion sessions.

(V) Seventy-two hour residential driver intervention programs

(1) Each seventy-two hour residential driver intervention program is to consist of at least twenty-one hours of alcohol and drug addiction programming that includes, at a minimum, all of the following:

(a) One hour of screening and individual;

(b) Fifteen hours of client education on alcohol and drug abuse and addiction including traffic safety education; and

(c) Five hours of small group discussion sessions.

(2) The program is to provide morning, mid-day, and evening meals of at least thirty minutes duration.

(W) Each driver intervention program is to have a written educational curriculum that includes, at a minimum, all of the following:

(1) Traffic safety education as it relates to alcohol and drug use;

(2) Client education on alcohol and drug abuse and addiction; and

(3) Small group discussion topics.

(X) Each program is to prepare an educational curriculum for its driver intervention. That program is to be approved by the program's governing authority and will include, at a minimum, all of the following:

(1) A timetable for conducting the program;

(2) An instructional outline for each topic/module; and

(3) A method of instuction, including audio/visual aids.

(Y) Each program is to have a program schedule that can be given to clients upon request.

(Z) Traffic safety education:

(1) Traffic safety education is to include, at a minimum, all of the following information on the criminal justice system and relevant law:

(a) Blood alcohol content (BAC), drugs, and impairment;

(b) Levels of license suspension and revocation; and

(c) Fines and levels of incarceration.

(2) The driving task and the specific psychomotor skills required.

(3) The physiological and psychological effects of alcohol and other drugs on driving performance.

(4) Traffic safety education sessions are not to exceed an instructor to client ratio of one to forty eight.

(AA) Client education on alcohol and drug abuse and addiction

(1) Client education on alcohol and drug abuse and addiction is to include, at a minimum, all of the following:

(a) Physical and psychological aspects of the use of alcohol and other drugs;

(b) Combining the use of alcohol with other drugs;

(c) Social consequences of the use of alcohol and other drugs;

(d) Signs and symptoms of abuse and dependence of alcohol and other drugs;

(e) Dysfunctional behavior resulting from the use of alcohol and other drugs;

(f) Progressive nature of alcohol and drug abuse and dependence;

(g) Abstinence as a lifestyle and self-help programs such as alcoholics anonymous and narcotics anonymous; and

(h) Treatment alternatives and local resources.

(2) Programming does not include the following:

(a) Individual and/or group counseling;

(b) Self-help study; or

(c) Anger management and stress reduction therapies.

(3) Staffing for client education group sessions on alcohol and drug abuse and addiction are not to exceed an instructor to client ratio of one to forty eight.

(BB) Indigent clients

(1) A driver intervention program that receives funds that originate and/or pass through the department is to have a policy and procedure that states that admission to the driver intervention program will not be denied because an individual is indigent as long as public funds are available to cover the cost of the program.

(2) Indigent individuals are persons who have in their possession at the beginning of a driver intervention program a document indicating the client is a recipient of public assistance, including, but not limited to, supplemental security income, social security disability income, medicaid, Ohio works first, or another category of assistance as defined by the department.

(3) The client record of indigent individuals is to include a photocopy of the documentation that was used to verify indigent status.

DRAFT – NOT FOR FILING

5122-29-13 SUD case management services.

(A) Substance use disorder (SUD) case management services means those activities provided to assist and support an individual in gaining access to medical, social, educational, and other services essential to meeting the client's basic human needs. Case management services may include interactions with the client's family members or other individuals or entities. Case management activities may include, but are not limited to, assessment, referral, monitoring, and follow-up.

(B) SUD case management services are to be delivered by individuals who are familiar with the community resources in which they provide the services.

(C) SUD case management services are to be provided according to a client's comprehensive individualized treatment plan developed in accordance with rule 5122-27-03 of the Administrative Code.

(D) SUD case management services do not include any of the following:

(1) Transportation;

(2) Waiting with an individual for appointments at social service agencies, court hearings, and similar activities;

(3) Direct services to which the client has been referred such as medical, educational, or social services; or

(4) Internal quality assurance activities, such as clinical supervisory activities and/or case reviews/staffing sessions.

DRAFT – NOT FOR FILING

5122-29-15 Peer support services.

(A) Peer support services are services for (1) individuals with a mental illness or substance us disorder, or both, who may also have a co-occurring developmental or intellectual disability and (2) the caregivers and families of such individuals.

(B) Peer support services consist of activities that promote resiliency and recovery, self-determination, advocacy, well-being, and skill development. Peer support services are individualized, resiliency and recovery focused, non-clinical, and based on increasing knowledge and skills through a peer relationship that supports an individual's or family's ability to address needs, navigate systems and promote recovery, resiliency, and wellness. They promote family driven, youth guided, trauma informed care and cultural humility, encourage partnership with individuals and families, and advocate for informed choice.

(C) As used in this rule:

(1) "Peer support services provider" means a provider, described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies, that is certified under this rule to provide peer support services.

(2) "Recovery" means the personal process of change in which an individual strives to improve their health and wellness, resiliency, and reach their full potential through self-directed actions.

(3) "Resiliency" means the ability to recover from setbacks, adapt well to change, and keep going in the face of adversity. It is the process of adapting well in the face of adversity, trauma, tragedy, threats, or significant sources of stress such as family and relationship problems, serious health problems, or workplace and financial stressors.

(4) "Wellness " means a broad approach for things individuals can do at their own pace, in their own time, and within their own abilities, that can help them feel better and live longer.

(D) Peer support services may include, but are not limited to:

(1) Ongoing exploration of recovery, resiliency, and wellness needs;

(2) Supporting individuals and their caregivers and families in achieving goals through increased knowledge, skills and connection as identified by the individual or family;

(3) Encouraging hope, empowerment, and personal agency in the recovery process;

(4) Supporting the development of life skills and practical daily functioning;

(5) Developing and working toward individualized recovery, resiliency, and wellness goals, while supporting multiple pathways;

(6) Modeling personal responsibility for resiliency, recovery, and lived experience to teach, mentor, and inspire recovery and wellness;

(7) Teaching and coaching skills to navigate behavioral health, social service, and community systems to effectively and efficiently utilize services;

(8) Assisting individuals, caregivers, and families in addressing and eliminating barriers to achieve and maintain recovery;

(9) Assisting with accessing, developing, and engaging natural supports, community resources, and formal or informal support systems;

(10) Promoting coordination and linkage among providers, systems, and community resources;

(11) Coordinating or assisting in crisis interventions and stabilization;

(12) Conducting outreach and community education to increase awareness and engagement;

(13) Attending and participating in team decision making or treatment team meetings;

(14) Planning, developing, and facilitating peer recovery group or peer-led recovery activities;

(15) Creating and updating recovery plans;

(16) Providing mentoring, mutual support, and guidance based on lived experience;

(17) Assisting individuals to engage in behavioral health services and overcome barriers to participation;

(18) Engaging additional supports that promote active participation in the individual's recovery; and

(19) Assisting individuals, caregivers, or families in the development of empowerment skills through advocacy and activities that mitigate discrimination and inspire hope.

(E) Providing services in a culturally inclusive and competent manner which includes not practicing, condoning, facilitating, or collaborating in any form of discrimination on the basis of ethnicity, race, gender, sexual orientation, age, religion, national origin, marital status, political belief, or mental or physical disability.

(F) Peer support services are not site specific but are to be provided in locations that meet the needs of the individual, caregiver, or families.

(G) Peer support services may be facilitated to individuals, families, or groups.

(H) Peer support services are to be provided by a person certified in accordance with in rule 5122-29-15.1 of the Administrative Code.

(I) Peer support services providers are to report to the department of behavioral health any events involving a certified peer supporter (whether employed or volunteering) that would

disqualify that certified peer supporter under rule 5122-29-15.1 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-16 Peer run organization service.

(A) As used in this rule:

(1) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(2) "Recovery" means the personal process of change in which an individual strives to improve their health, wellness, and resiliency, as well as reach their full potential through self-directed actions.

(B) Peer run organization service is comprised of both activities specified in paragraph (C)(1) of this rule and one or more activities specified in paragraph (C)(2) of this rule that are delivered by individuals, a majority of whom have direct lived experience with mental illness or a substance use disorder, that leverages the power of those individuals' shared lived experiences to promote the behavioral health recovery and well-being of other persons with mental illness or a substance use disorder.

(C) For a provider to be certified to provide peer run organization ed service, the provider is to demonstrate all of the following:

(1) That it is able to provide both of the following activities of the peer run organization service:

(a) Recovery supports as defined in section 5119.01 of the Revised Code, whether occurring in an individual or group setting; and

(b) Peer support services provided by certified peer supporters, as defined in rule 5122-29-15 of the Administrative Code, that are not delivered as part of a comprehensive individualized treatment plan described in rule 5122-27-03 or reimbursed on a fee-for service or other third-party reimbursement basis through medicaid or private insurance. Receipt by the provider of a grant, contract, appropriation, or other public funding to support peer run organization services does not constitute reimbursement for purposes of this paragraph.

(2) That it is able to provide one or more of the following activities of the peer run organization service:

(a) Consultation to a board of alcohol, drug addiction, and mental health services and other community agencies regarding identifying unmet recovery needs and potential solutions for fulfilling those needs;

(b) Recovery planning;

(c) Recovery meetings;

(d) Support groups;

(e) Links to resources;

(f) Skills-building groups;

(g) Increasing recovery supports workforce capacity through training, development, and other activities;

(h) Harm reduction activities that may be supported with funding from the United States substance abuse and mental health services administration (including overdose prevention and response services) according to guidance from that federal agency;

(i) Education;

(j) Socialization activities;

(k) Wellness activities;

(l) Activities involving art;

(m) Meals;

(n) Health and fitness classes;

(o) Structured volunteer and work activities;

(p) Recovery-focused outreach and advocacy activities; or

(q) Hosting of recovery celebrations.

(3) That either of the following is true:

(a) At least fifty-one per cent of the individual members of the provider's board of directors or other governing body self-identify as having direct, lived experience and are in recovery from a mental illness or substance use disorder; or

(b) In the case of a provider whose primary mission is the delivery of family peer support services or services focused on the caregiver experience, at least fifty-one per cent of the individual members of the provider's board of directors or other governing body self-identify as caregivers of an individual with mental illness or a substance use disorder.

(4) A provider must establish, publish, and generally maintain predictable hours appropriate to its service model and the needs of the community it serves.

(5) That the provider makes the service available to all individuals in the community with mental illness or substance use disorder, not restricting the service to participants or residents of any particular housing program, treatment program, service agency, or residential facility, and that all of the following are the case:

(a) The provider's outreach and promotional materials clearly state that the activities of the service are open to all community members;

(b) The provider has no referral, enrollment, or eligibility standards limiting access to individuals connected to a specific housing program, treatment provider, service agency, or residential facility; and

(c) The provider's policies and handbook do not impose restrictive eligibility criteria tied to particular housing, treatment, or service providers.

For purposes of demonstrating compliance with this paragraph, a provider is to maintain outreach materials or web site statements, internal policy documents, and attendance or intake records that reflect broad community participation rather that limitation of activities to a specific housing program, treatment provider, service agency, or residential facility.

(D) In addition to having at least one physical site as required in chapter 5122-25, a provider of peer run organization service may use one or more mobile units intended to meet the needs of underserved populations.

(1) A mobile unit is to be a van, larger recreational vehicle, or other similar type of larger motor vehicle that is equipped with enough space for at least two adults to meet comfortably for the provision and receipt of peer support services. The van, larger recreational vehicle, or similar type of larger motor vehicle is not to be owned or rented by a member of the provider's staff, governing board, or advisory board. A mobile unit is not to be a motorcycle or bicycle.

(2) A mobile unit is to possess a valid license plate. A copy of the unit's vehicle registration is to be on file at the provider's physical location listed on the provider's application filed under rule 5122-25-02, 5122-25-03, or 5122-25-04 of the Administrative Code.

(E) A provider of peer run organization service that provides peer support services, as described in paragraph (C)(1)(b) of this rule, may use telehealth for the delivery of the peer support services in accordance with rule 5122-26-22 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-17 Psychosocial rehabilitation service (PSR).

(A) As used in this rule:

(1) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(2) "Collateral contact" occurs when the provider contacts individuals who play a significant role in the service recipient's life.

(B) Psychosocial rehabilitation (PSR) service

Psychosocial rehabilitation (PSR) service is a service comprised of one or more of the mental health activities specified in paragraph (C) of this rule. For each individual who receives the service, the service is to be tailored to that individual's needs consistent with their comprehensive individualized treatment plan developed under rule 5122-27-03 of the Administrative Code.

The service is intended for individuals with a mental health diagnosis resulting in functional deficits who need intensive community support to maintain stability and achieve recovery goals. It is intended to restore, rehabilitate, and support activities and routines of daily functioning within the individual's natural community environment by addressing impairments and barriers arising from and impacting the individual's mental illness. The service may be delivered at a fixed site, an individual's home, or in the community; the place of services delivery is to be based on the needs of the particular individual served.

(C) For a provider to be certified to provide PSR service, the provider is to demonstrate all of the following:

(1) That it is able to provide all of the following activities of the service:

(a) Ongoing assessment of needs

The provider is able to complete regular assessments of an individual's needs and barriers and, after each assessment, review the individual's comprehensive individualized treatment plan and document review results in accordance with rule 5122-27-03 of the Administrative Code.

(b) Assistance in achieving personal independence in managing basic needs

The provider is able to support an individual with independently managing their identified basic needs and life skills directly related to their mental illness, with progress measured by timeframes determined in the individual's comprehensive individualized treatment plan that reflect the individual's mental health-related functional goals.

(c) Facilitation of further development of daily living skills

The provider is able to facilitate development of an individual's identified daily living skills, directly related to the individual's mental illness, through structured practice and coaching until the individual meets the comprehensive individualized treatment plan goals that reflect their mental health-related functional needs.

(d) Coordination of the comprehensive individualized treatment plan

The provider is able to coordinate implementation of an individual's comprehensive individualized treatment plan by doing all of the following:

(i) Ensuring that services identified in the plan are initiated and maintained by completing collateral contact and documenting the individual's engagement and progress;

(ii) Assisting with the identification of and engagement with an individual's natural supports, such as family and peers; and

(iii) Establishing linkages to formal community services/systems through collateral contact identified in an individual's comprehensive individualized treatment plan.

(e) Symptom monitoring

The provider is able to monitor the individual's psychiatric and/or substance use symptoms, review trends, and adjust coping strategies and comprehensive individualized treatment plan interventions to achieve stabilization within the community.

(f) Coordination and/or assistance in crisis management and stabilization as needed

Through collateral contact and direct service, the provider is able to coordinate crisis management and stabilization of an individual by implementing a safety plan, contacting crisis resources, and conducting post-crisis follow-up with the individual to reduce crisis frequency and severity and maintain community safety.

(g) Education and training

The provider is able to deliver tailored psychoeducation and skills training on identified topics and verify learning through demonstrated skills and real-world application linked to comprehensive individualized treatment plan goals.

(h) Mental health interventions

The provider is able to deliver targeted mental health interventions to reduce symptoms, behaviors, and thought patterns that interfere with achieving comprehensive individualized treatment goals. Intervention progress may be tracked by attendance, retention, symptom reduction, and/or functional gains.

(i) Positive impact activities

The provider is able to use positive impact and community functioning interventions such as, but not limited to, individualized education, modeling, role-play, community-based practice, problem-solving coaching, and prompting to help the individual recognize how their mental illness affects their community functioning, select and implement appropriate settings, initiate pro-social actions, and evaluate the outcome of each intervention as identified in the comprehensive individualized treatment plan.

(2) That it is able to deliver the service to groups or individuals;

(3) That it is able to deliver the service face-to-face, by telephone, or by video conferencing;

(4) That regarding staffing, it meets all of the following criteria:

(a) Subject to paragraph (D) of this rule, has an interdisciplinary team of staff qualified according to rule 5122-29-30 of the Administrative Code to provide and supervise the service;

(b) Ensures that each particular staff member on the interdisciplinary team provides only those components of the service for which the individual possesses the appropriate qualifications or credential;

(c) Ensures that when an individual served is enrolled in residential treatment or residential support facility setting, the service is provided by staff who are organized, distinct, and separate from the residential service as evidenced by staff job descriptions, time allocations or schedules, and development of service rates; and

(d) Ensures that each individual to be served is assigned one staff member, based on the specific needs of the individual, who is the designated person to provide case coordination.

(5) That it has a staff development plan that is based on identified individual needs of staff providing the PSR service. Evidence that the plan is being followed is to be maintained. The plan is to address, at a minimum, all of the following:

(a) An understanding of systems of care, such as natural support systems, entitlements and benefits, inter-agency and intra-agency systems of care, crisis response systems and their purpose, and the intent and activities of the service;

(b) Characteristics of the population to be served, such as psychiatric symptoms, medications, culture, and age/gender development; and

(c) Knowledge of the service's purpose, intent, and activities.

(D) A practitioner who possesses a high school diploma and specific training related to persons with mental health conditions or needs may perform the activities of the PSR service specified in paragraphs (C)(1)(b), (C)(1)(c), and (C)(1)(i) of this rule.

(E) While, pursuant to paragraph (C)(1) of this rule, a provider is to demonstrate the ability to provide all activities of the PSR service to be certified for the service, a provider may contract with another provider certified under this rule to provide one or more activities of the service specified in that paragraph.

DRAFT – NOT FOR FILING

5122-29-18 Therapeutic behavioral services.

(A) As used in this rule,

(1) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(2) "Collateral contact" occurs when the provider contacts individuals who play a significant role in the service recipient's life.

(B) Therapeutic behavioral services are intensive, one-on-one or group outpatient services for adults with a mental health diagnosis. They are supports and solution-focused interventions, delivered on a short-term basis, intended to achieve identified goals or objectives in an individual's comprehensive individualized treatment plan developed under rule 5122-27-03 of the Administrative Code and focused on reducing high-risk, disruptive, and/or self-destructive behaviors by building specific behavioral and emotional management skills. The service may be delivered in all settings that meet the needs of the individual.

(C) For a provider to be certified to provide therapeutic behavioral services, the provider is to demonstrate all of the following:

(1) That it is able to provide all of the following activities of therapeutic behavioral services:

(a) Collateral contact

The provider is able to do both of the following:

(i) Consult with one or more eligible providers of the services, as specified in paragraph (C)(3) of this rule, for the purpose of addressing an individual's identified needs and participating in service planning. Such consultation may include planning for individualized supports and coordinating healthcare, behavioral healthcare, and non healthcare services in accordance with the individual's comprehensive individualized treatment plan; and

(ii) Refer an individual to other healthcare, behavioral healthcare and non-healthcare services to avoid more restrictive levels of treatment.

(b) Use of evidence-based techniques

The provider is able to provide solution-focused interventions and emotional and behavioral management for an individual, drawn from evidence-based modalities to ameliorate targeted symptoms and improve the individual's capacity to cope with or prevent symptom manifestation.

(c) Treatment planning

With the individual's participation and consent, the provider is able to utilize strengths-based treatment planning including assisting the individual or individual's support systems (e.g., a parent, guardian, or other treatment professionals involved with the individual's care) in identifying needs and treatment goals associated with the individual's mental illness or behavioral health condition.

(d) Restoration of self-management skills social skills and daily functioning

The provider is able to provide therapeutic interventions designed to promote safe and adaptive alternatives to high-risk, disruptive, and/or self-destructive behaviors resulting in improved emotional and behavioral self-regulation and functioning; improved insight and self-awareness; increased community tenure; and effective functioning to support the individual's ability to maintain safety within the individual's natural environment as related to their diagnosed mental illness or behavioral health condition.

(e) Crisis prevention and amelioration

The provider is able to provide therapeutic support to assist an individual and the individual's support systems to identify potential psychiatric or personal crises, develop crisis management plans, and, as appropriate, provide collateral contact to restore stability and functioning to avoid more restrictive levels of treatment.

(2) That for each individual to receive the services, it is able to monitor reductions or increases in symptom severity, the need for transition to a higher or lower level of care, and increases or decreases in social skills and daily functioning. Monitoring is needed to detect early signs of clinical deterioration, initiate timely intervention, evaluate response to treatment, adjust the comprehensive individualized treatment plan, and support the individual's recovery goals by strengthening coping skills, community participation, and independence in daily living.

(3) That it employs or contracts with either or both of the following to provide the services:

(a) One or more behavioral health practitioners who possess, at a minimum, a bachelor's or master's degree in social work, psychology, nursing, or in a related human services discipline; or

(b) One or more qualified behavioral health specialists, as defined in rule 5122-29-30 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-19 Consultation service.

(A) As used in this rule, "provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) "Consultation service" means a formal and systematic information exchange between a provider and a community partner (including, but not limited to, a school system, court, family member of an individual receiving or to receive behavioral health services, or other person who is not an established provider of the individual receiving or to receive behavioral health services) that is directed towards the sharing of information on how to support, intervene, and address concerns or behaviors of the individual receiving or to receive behavioral health services, including facilitating the availability of appropriate behavioral health services for that individual.

(C) Consultation may be focused on the clinical condition of an individual served by another system or focused on the functioning and dynamics of another system. Consultation related to the clinical condition of an individual served is to be provided by staff qualified according to paragraph (D) of this rule.

(1) The provider is to periodically survey other community systems to determine behavioral health consultation needs that may be desired by the systems, individuals, or families being served by those other systems

(2) The provider is to maintain a record of all consultation services provided, including the name of the individual or system to whom the service was provided, the nature of the consultation, and the outcome of the consultation.

(D) Consultation service is to be provided and supervised by staff who are qualified according to rule 5122-29-30 of the Administrative Code.

DRAFT – NOT FOR FILING

5122-29-22 Referral and information service.

(A) As used in this rule, "provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) "Referral and information service" means responses, usually by telephone, to inquiries from individuals about services in the community. Referral may include contacting any agency or a provider to secure services for the individual requesting assistance.

(C) Referral and information service is to be planned and coordinated with other health and human service providers, and is to satisfy both of the following conditions:

(1) Have a mechanism to compile information about services available in the service system and community; and

(2) Have processes to determine whether individuals referred were able to access services, were satisfied with the services, or experienced any problems with the referral source. This information is to be used to determine if particular providers are to continue to be used as referrals for individuals seeking services. All state and federal confidentiality laws are to be adhered to in this process.

(D) The provider is to ensure access and availability of referral and information service including satisfying both of the following conditions:

(1) Having a published telephone number for the referral and information service, including a published telephone number for special telephone services for the hearing impaired; and

(2) Ensuring access and availability to the referral and information service for individuals whose primary means of communication is a language other than English, and for persons with communication impairments (such as speech, language, or hearing disorders, access to telecommunication relay services (TRS)), and for persons with visual impairments. With respect to individuals with communication impairments, the provider is to ensure access to a telephone communication relay service (TRS). The services associated with TRS include, but are not limited to, text to speech relay and signing to speech relay.

(E) Each call and contact is to be logged and include the date, time, and person answering the call or contact.

(F) A referral and information service is not a hotline service, and is not intended to replace the crisis assistance function of a hotline service.

DRAFT – NOT FOR FILING

5122-29-27 Supplemental behavioral health services.

(A) As used in this rule:

(1) "Mental illness" has the same meaning as in section 5119.01 of the Revised Code.

(2) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) "Supplemental behavioral health services" means services other than those specifically listed in this chapter. Supplemental behavioral health services may include representative payeeship, outreach, screening, education, and other supportive behavioral health services and may be offered by a variety of entities, including YMCAs, churches, or county family and children first councils.

(C) A provider applying for initial or renewal certification of supplemental behavioral health services under rule 5122-25-02 or 5122-25-04 of the Administrative Code, or seeking to add supplemental behavioral health services to the menu of services the provider provides under rule 5122-25-03 of the Administrative Code, is to include all of the following information in their application:

(1) The name of each specific supplemental behavioral health service to be provided along with a brief description of each specific service; and

(2) A letter from the board of alcohol, drug addiction, and mental health services of each alcohol, drug addiction, and mental health service district where the provider intends to provide supplemental behavioral health services that describes the board's support for the provider's provision of the services.

(D) A provider of supplemental behavioral health services is to do all of the following:

(1) Ensure that the provision of the services meets the appropriate standards or regulations;

(2) Ensure that staff providing the services have participated in orientation or training regarding basic information about mental illness, emotional disturbance, and substance use disorders, and know how to obtain assistance from the behavioral health system if needed; and

(3) Develop mechanisms to solicit and receive feedback about the quality of the services from individuals served.

DRAFT – NOT FOR FILING

5122-29-28 Intensive home based treatment (IHBT) service.

(A) In addition to the definitions in rule 5122-24-01 of the Administrative Code, the following definitions apply to this rule:

(1) "Caseload" means the individual cases open or assigned to each full-time equivalent IHBT staff member.

(2) "Continued stay review" means a review of a child/adolescent's functioning to determine the need for further services to achieve or maintain service goals and objectives.

(3) "Crisis response" means the immediate access and availability, as clinically indicated, to the child/adolescent and family, which may include crisis stabilization services in accordance with rule 5122-29-10 of the Administrative Code, safety planning, and the alleviation of the presenting crisis.

(4) "Family" means any individual or caregiver related by blood or affinity whose close association with the child/adolescent is the equivalent of a family relationship as identified by the child/adolescent, including kinship and foster care.

(5) "Home" means any family living arrangement including but not limited to biological, kinship, adoptive, foster home, and non-custodial families who have made a commitment to the child/adolescent.

(6) "Out-of-home placement" means any removal of the child/adolescent from his or her home. Planned respite, where the child's or adolescent's main residence remains their home, is not considered out-of-home placement.

(7) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) Intensive home based treatment (IHBT) service is a comprehensive behavioral health service provided to a child/adolescent with serious emotional disturbance (SED) and their family, designed to treat mental health conditions that significantly impair functioning. IHBT may also be utilized for the treatment of children and adolescents that have co-occurring substance use or neurodevelopmental needs, when these needs co-occur with a mental health condition. IHBT is provided for the purpose of preventing out of home placement or facilitating a successful transition back home. IHBT integrates trauma-informed and resilience-focused assessment, crisis response, individual and family psychotherapy, service and resource coordination, and rehabilitative skill development with the goal of either preventing the out-of-home placement or facilitating a successful transition back to home. These intensive, time-limited behavioral health services are provided in the child/adolescent's natural environment with the purpose of stabilizing and improving their behavioral health functioning as documented using the Ohio specific child and adolescent needs and strengths (CANS) tool.

The purpose of IHBT is to enable a child/adolescent with SED to function successfully in the least restrictive, most normative environment. IHBT services are culturally, ethnically, racially, developmentally and linguistically appropriate, and respect and build on the strengths of the child/adolescent and family's race, culture, and ethnicity.

(C) The following describes the activities and components of IHBT:

(1) IHBT is an intensive service that consists of multiple in person contacts per week with the child/adolescent and family, which includes collateral contacts related to the behavioral health needs of the child/adolescent as documented in the individual client record (ICR) as mandated by Chapter 5122-27 of the Administrative Code. IHBT can be provided via telehealth in accordance with rule 5122-26-22 of the Administrative Code.

(2) IHBT is provided in the home, school, and community where the child/adolescent lives and functions;

(3) The frequency and modality of contacts may fluctuate based on the assessed needs and unique circumstances of the child, adolescent, and family;

(4) IHBT is strength-based and family-driven, with both the child/adolescent and family regarded as equal partners with the IHBT staff in all aspects of developing the service plan and service delivery;

(5) IHBT is provided by staff with a caseload that averages over any six month period and per full time equivalent staff:

(a) Twelve or less when provided by a team of two,; or

(b) Six or less when provided by an individual staff member.

(6) Immediate crisis response is available twenty-four hours a day, seven days a week by the lead IHBT team member with back-up coverage available from other IHBT team members or the IHBT team supervisor.

(7) Each child/adolescent and family receiving IHBT is assessed for risk and safety issues. A jointly written crisis and safety plan is to be developed and provided to the child/adolescent and family.

(8) Collaboration is mandated to be performed with other child-serving agencies or systems, e.g., school, court, developmental disabilities, child welfare, and health care providers that are providing services to the child/adolescent and family, as well as family and community supports identified by the child/adolescent and family.

(9) The service activities and components are individually tailored to meet the needs of the child/adolescent and family. Appointments are made at a time that is convenient to the child/adolescent and family, including evenings and weekends if necessary.

(10) The service is time-limited, with length of stay matched to the presenting behavioral health needs of the child/adolescent and the family.

(11) The IHBT team will collaboratively develop a plan to transition with each youth and family. The plan will include a focus on transition to other services, supports and providers for services and supports based on the individualized needs of the youth and family.

(D) Eligibility for IHBT will be determined by the IHBT team in collaboration with the youth and family and other cross systems partners by documenting the following criteria:

(1) Is clinically determined to meet the "individual with serious emotional disturbance" (SED) criteria in rule 5122-24-01 of the Administrative Code and the child or adolescent;

(a) Is under twenty-one years of age;

(b) Has a mental health need;

(c) Has an Ohio specific CANS assessment that indicates marked to severe behavioral/emotional impairment and at least one of the following:

(i) Impairment that seriously disrupts life functioning; or

(ii) Risk behaviors that are rated as actionable on the CANS.

(2) Meets one or more of the following criteria as documented in the ICR:

(a) Is at risk for out-of-home placement due to their behavioral health conditions;

(b) Has returned within the previous thirty days from an out-of-home placement or is transitioning back to their home within thirty days; or

(c) Needs a high intensity of behavioral health interventions to safely remain in or return home.

(E) The provider is to demonstrate that the following staff standards and qualifications are met:

(1) A minimum of two full-time equivalent staff members provide the service. Services may be provided by a single individual or team of staff clearly sharing various responsibilities for the same child/adolescent and family. Each child/adolescent is to have a staff assigned with lead responsibility.

(2) The provider is to have a documented plan for clinical supervision of each team member.

(3) The IHBT supervisor is to have primary responsibility for providing supervision to the IHBT staff twenty-four hours a day, seven days a week. If the IHBT supervisor is unavailable, then supervision is to be provided by staff qualified according to rule 5122-29-30 of the Administrative Code.

(F) The provider is to demonstrate that each IHBT staff has an individualized training plan based on an assessment of their specific training needs. The following professional training and development criteria are to be met:

(1) Each staff member receives an assessment of initial training needs based on the skills and competencies necessary to provide IHBT service prior to providing IHBT service; and

(2) The agency is to have a written description of the skills and competencies necessary to provide IHBT service, which include, at a minimum, the following:

(a) Family systems;

(b) Risk assessment, crisis stabilization, and safety planning;

(c) Parenting skills and supports for children/adolescents with SED;

(d) Cultural competency;

(e) Intersystem collaboration with a focus on schools, courts, and child welfare:

(i) Knowledge of other systems;

(ii) System advocacy; and

(iii) Roles, responsibilities, and mandates of other child/adolescent-serving entities;

(f) Trauma-informed and resiliency-focused care;

(g) Educational and vocational functioning:

(i) Assessment and intervention strategies for resolving barriers to successful educational and vocational functioning;

(ii) Knowledge of special education laws; and

(iii) Strategies for developing positive home-school partnerships and connections;

(h) IHBT philosophy, including strength-based assessment and treatment planning; and

(i) Understanding the complex and interconnected range of symptoms and needs of children and adolescents, including co-occurring substance use disorders and developmental disabilities.

(G) The provider's training plan is to include provisions for ongoing training specific to the identified training needs of the staff as it relates to the population served, including attention to cultural competency, changing demographics, new knowledge or research, and other areas identified by the agency.

(H) The provider is to demonstrate that each IHBT supervisor receives training specific to the clinical and administrative supervision of the service.

(I) The provider is to obtain satisfactory fidelity reviews based on the provider's specific program modality every twelve months by an individual or organization external to the provider and designated by the department, utilizing the IHBT individual provider model fidelity rating tool, version March 1, 2022, or the IHBT teamed-model fidelity rating tool, version March 1, 2022, available at ihbtohio.org, or be licensed by department-approved evidence-based practice (EBP). The provider is to incorporate the results of the fidelity review into the provider's performance improvement program, if indicated.

(J) Intensive home based treatment service is to be supervised by staff who are qualified according to rule 5122-29-30 of the Administrative Code.

(K) IHBT is to be provided by persons with competency in the provision of mental health interventions through one of the following program configurations:

(1) At least one licensed practitioner and at least one other licensed practitioner who is authorized to provide services pursuant to rule 5122-29-30 of the Administrative Code and who are providing an evidence-based practice approved by the department and are working in a program licensed by a national accreditation body or their delegate. Each practitioner is to have their own caseload of clients.

For those providers who are delivering functional family therapy (FFT), the services may be delivered by an individual who is licensed to provide services pursuant to rule 5122-29-30 of the Administrative Code;

(2) At least two or more licensed or licensed-eligible practitioners who are eligible to provide services pursuant to rule 5122-29-30 of the Administrative Code and who are providing an evidence-supported practice approved by the department. Each practitioner must have their own caseload of clients; or

(3) At least two practitioners eligible to provide services pursuant to rule 5122-29-30 of the Administrative Code. One of the practitioners is to be licensed and the other either a qualified behavioral health specialist as defined in rule 5122-29-30 of the Administrative Code or a peer supporter who holds a valid and unrestricted certification from the department issued in accordance with rule 5122-29-15.1 of the Administrative Code. The peer supporter is to be a family peer supporter or a youth peer supporter in accordance with rule 5122-29-15.1 of the Administrative Code. Peer supporters will also demonstrate competency working with children or adolescents with SED and their families. These practitioners are to share a caseload of clients.

(L) A provider of FFT who provides the service in accordance with the national evidence based model, found at https://www.fftllc.com/about-fft-training/clinical-model.html, does not need to meet the standards of paragraphs (C) and (E) to (H) of this rule. Any provider of FFT without meeting all other standards of this rule will be certified as "IHBT-FFT Only."

DRAFT – NOT FOR FILING

5122-29-29 Assertive community treatment (ACT) services.

(A) As used in this rule:

(1) "ACT certified provider" means a provider that is certified under this rule to provide assertive community treatment services.

(2) "ACT team" means a multidisciplinary team of practitioners, all of whom are qualified in accordance with rule 5122-29-30 of the Administrative Code and have specialty training in the provision of assertive community treatment, who provide team-based services in accordance with the ACT model and are employees or contractors of an ACT certified provider.

(3) "Approved fidelity review" means a fidelity review conducted by an independent validation entity recognized by the department.

(4) "Provider" means a provider described in rule 5122-25-01 of the Administrative Code to whom Chapter 5122-25 of the Administrative Code applies.

(B) Assertive community treatment (ACT) services are intended for individuals diagnosed with a severe and persistent mental illness that need high service intensity. ACT services are also provided to the family members and other support systems of such individuals. Individuals receiving ACT services may also have a co-occurring diagnosis.

(C) The purpose of ACT services is to provide the necessary interventions and support to maximize recovery and promote success in employment, housing, and stability in the community. The service is available twenty-four hours a day, seven days a week.

(D) ACT services providers are to employ one or more ACT teams to implement one or more programs that meet the minimum fidelity criteria described in this rule using the tool for measurement of assertive community treatment (TMACT) or the dartmouth assertive community treatment scale (DACTS).

(E) Before submitting an application for initial certification under this rule, a provider is to undergo an approved fidelity review and achieve a minimum fidelity score of 3.0.

(F) Once a provider becomes an ACT certified provider, each of its ACT teams is to undergo ongoing approved fidelity reviews. Each fidelity review is to occur within twelve months of the team's most recent fidelity review.

(G) Within three years of each ACT team's initial approved fidelity review, the team is to achieve a minimum fidelity score of 4.0. If a team fails to achieve that standard or fails to achieve a minimum fidelity score of 4.0 at any fidelity review conducted after the three-year period, the provider responsible for the ACT team is do one of the following:

(1) Submit a waiver request to the department in accordance with rule 5122-25-10 of the Administrative Code; or

(2) Disband the ACT team because of fidelity noncompliance.

(H) As part of a waiver request submitted under paragraph (G)(1) of this rule, the provider responsible for the ACT team is to submit all of the following:

(1) A copy of the ACT team's most recent fidelity review report.

(2) A proposed timeframe by which the ACT team will achieve the minimum 4.0 fidelity score.

(3) A fidelity action plan the ACT team has developed in collaboration with an independent validation entity recognized by the department that addresses all problem areas identified in the most recent fidelity review report.

(4) Documentation that the provider has established an internal steering committee responsible for guiding and monitoring the implementation of the fidelity action plan described in paragraph (H)(3) of this rule. The committee is to include, at a minimum, the provider's ACT team leader, clinical director, and quality improvement or quality assurance director.

(5) If at the time the waiver is requested the ACT team is subject to a previously-granted waiver, documentation that that ACT team has undergone a new fidelity review prior to the expiration of the previously-issued waiver and a copy of the results of that review.

(I) If an ACT team has developed a fidelity action plan described in paragraph (H)(3) of this rule, the provider responsible for the ACT team is to (a) ensure that plan is addressed in the provider's quality improvement plan developed under rule 5122-28-03 of the Administrative Code and (b) monitor progress toward achieving plan objectives.

(J) The department may request that a certified ACT provider undergo a new fidelity review at any time based on one or more findings of noncompliance with this chapter.

(K) Prior to termination of ACT services due to an individual's unexpected non-participation, an ACT team will attempt at least two contacts per month with the individual until one of the following is the case:

(1) The ACT team has attempted to contact the individual on at least two occasions each month for a period of ninety days and the individual has not responded. An ACT team is to document each attempt at contacting an individual in the individual's record; or

(2) The individual notifies the ACT team of the individual's desire to discontinue the services.

(L) ACT services are to be provided and supervised by staff who are qualified according to rule 5122-29-30 of the Administrative Code. A particular staff member may provide only those components of the service for which the individual possesses the appropriate qualifications or credential.